✦ Watch the Work · Free template

# The Healthcare *Value Stream Map.*

Map one workflow on one page. Circle the waste. Find the one constraint worth fixing, and see whether AI actually fits.

45–60 minutes | One workflow | Pen and paper is fine

**01** **Pick one workflow, start to finish**

Something that repeats every day and frustrates people: referral intake, prior auth, scheduling, refills, results follow-up. Write down the trigger (what starts it) and the finish line (what "done" means).

**02** **Watch the work, don't ask about it**

Sit next to the person who does it. Follow one real case end to end. What people tell you in a meeting and what actually happens are rarely the same.

**03** **Fill in the map (page 2)**

One column per step. For each: who does it, what system they use, how long the hands-on work takes, how long it waits before the next step, and every workaround you see.

**04** **Circle the waste and find the constraint (page 3)**

Walk the checklist. The constraint is the step where work piles up, or the biggest wait. Fixing anything else just moves the pile.

**05** **Ask whether AI fits, and only then**

Some constraints need a process change, not software. The AI-fit check tells you which is which. Page 4 shows a full example.

## Three rules from the floor

- **Map what is, not what should be.** The policy binder is not the workflow.
- **Count the waiting.** In most healthcare workflows, the work takes minutes and the waiting takes days.
- **Every process ends with a person.** Write down who feels the delay: the patient, the nurse, the physician.

✦ Step 03 · Current state

# Map the workflow *as it really runs.*

One column per step, left to right. Use pencil. Add a second sheet if you run out of columns; that usually means you found the problem.

WORKFLOW

TRIGGER (WHAT STARTS IT)

DONE WHEN…

| Step | 1 | 2 | 3 | 4 | 5 | 6 |
|---|---|---|---|---|---|---|
| **What happens** verb + object | | | | | | |
| **Who does it** role, not name | | | | | | |
| **System / tool** EHR, fax, phone, portal, paper | | | | | | |
| **Workarounds** sticky notes, re-typing, calls | | | | | | |
| **% sent back** rework rate | | | | | | |
| **Wait before (days/hrs)** | | | | | | |
| **Hands-on time (min)** | | | | | | |

**Total wait:** **Total work:** **Work ÷ total:** %

**Lead time** = total wait + total work **Work ÷ total** under 10% is normal in healthcare. It's also where the opportunity is. ★ Star the step with the biggest pile or the longest wait.

✦ Step 04 · Circle the waste

# Walk the map. *Tick what you saw.*

Eight kinds of waste, translated for healthcare. Mark the step number next to each one you find.

- [ ] **Waiting**
  Work sits in a queue, an inbox, a fax tray or a voicemail.
- [ ] **Handoffs**
  Work changes hands, and context gets lost each time.
- [ ] **Re-entry**
  Same data typed into two or more systems.
- [ ] **Rework**
  Denials, missing info, call-backs, "please resend."
- [ ] **Searching**
  Hunting for a chart, a fax, a form, an answer.
- [ ] **Backlog**
  Piles of unworked items: referrals, messages, results.
- [ ] **Swivel-chair**
  Clicking between screens to stitch one task together.
- [ ] **Wasted talent**
  Nurses and physicians doing clerical work.

FIND THE CONSTRAINT

## Which step limits the whole flow?

- [ ] Where does work pile up?
- [ ] Where is the longest wait on the ladder?
- [ ] Where does work get sent back most?
- [ ] If this step were instant, would the whole workflow speed up?

My constraint is step #

_____

AI-FIT CHECK

## Is AI the right fix for this step?

- [ ] It happens many times a day, the same way.
- [ ] The inputs are digital, or could be (fax, PDF, form, call).
- [ ] A person could explain the rules in an afternoon.
- [ ] A human can review the output before it matters.
- [ ] The fix lives inside the tools the team already uses.

4–5 ticks: strong AI fit. 2–3: fix the process first. 0–1: it's not an AI problem.

FUTURE STATE

## If the constraint were fixed, the workflow would look like…

_____

_____

_____

✦ Worked example

# Prior authorization *for an imaging order.*

A typical current-state map of what we see in practices. Your numbers will differ; the shape usually doesn't.

**1 · Order arrives**
Referral faxed in. Front desk scans and indexes it.
wait 1 day · 8 min

**2 · Check if auth needed**
Staff look up payer rules on 3 portals.
wait 1 day · 12 min

THE CONSTRAINT

**3 · Build the request**
Re-type demographics, hunt clinicals, call the office for missing notes.
wait 3 days · 25 min

**4 · Submit + follow up**
Portal or fax. Call payer for status.
wait 4 days · 15 min

**5 · Schedule patient**
Call the patient. Often 2–3 attempts.
wait 1 day · 10 min

**10 days**
lead time from order to scheduled

**70 min**
of actual hands-on work

**<1%**
of the lead time is work

WASTE FOUND

Re-entry (step 3), searching for clinicals (3), rework when payers ask for more (4), waiting at every handoff, and a growing backlog behind step 3.

AI-FIT: 5 / 5

High volume, digital inputs, clear payer rules, staff review before submit, and it can live inside the current queue. AI drafts the request; a person checks and sends it.

*Illustrative example based on common patterns, not a specific client.*

✦ Want a second set of eyes?

## See the 3 workflows *we'd fix first.*

Type your practice name into the Workflow Grader. It takes 5 seconds. Then, if you want, I'll build a free working demo of #1 around your real workflow. No connection to your systems needed.

Grade my workflow →

marcoslacayobosche.com/diagnose · I build 4 free demos a month.